



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه عبد القادر علي محمد

العنوان : بليس

الحي / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 28309251303688 رقم التليفون : 01003866240

العمر (تاريخ الميلاد) : 25-09-1983 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج :
الأورام

خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمه عبد القادر علي محمد National ID: 28309251303688	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Contrast-enhanced MRI of the brain and orbits
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Pathology Lab
معمل الباثولوجي
Pathology request form

*Patient Name:

فاطمة عبدالقادر

*Hospital Number:

*Date of Birth:

*Sex:

Male

☐

Female

☐

*Requesting Physician:

A Case of Ca breast

*Department:

*Clinical data:

for ER, PR, Her2 & Ki67

Specimen data:

5g

Number Of specimens:

2/1
CCT

*Time Taken

*Time into Fixative

1.	:	:
2.	:	:
3.	:	:
4.	:	:
5.	:	:

Sent by

Department/Site

Number of
Containers
()

Lab Use Only

Demographics Checked	Initials
Received number of Containers ()	
Accessioning	
Grossing	
Embedding/Microtomy:	
Pathologist	

Time out of Formalin

Time submitted to
pathologist

*Must be filled.

Issue date: 9/3/2021

Version No.: 2/0

Form code: F/SOP/PI/20/01

جمهورية مصر العربية
بمطابقة كود رقم الشخصية

فاطمة
عبدالقادر على محمد
بساتين بركات
مركز بلبيس - الشرقية

٢٨٣.٩٢٥١٣.٣٦٨٨

JY8377811

٠١٠٣٨٦٦٤٤٠

٢٠٢٣/٠٤ ٢٨٣.٩٢٥١٣.٣٦٨٨

حاصله على دبلوم تجاره

انثى مسلمة متزوجة
احمد عبدالفتاح سيف خليفه
البطاقة سارية حتى ٢٠٣٠/٠٤/٢٩

٨

Name: فاطمة عبد القادر علي محمد		
MR. : 8279		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انها The patient was initially diagnosed with hormone-receptor-positive, HER2-positive breast cancer and received neoadjuvant chemotherapy with the TCHP regimen followed by breast-conserving surgery, achieving a clinical response. Post-operative therapy included phasgo, femara, and zoladex continued until December 2025. Subsequent imaging identified brain metastases and skin nodules on the right breast; a biopsy of the brain lesions confirmed metastatic adenocarcinoma of breast origin (ER 8/8, PR 0/8, HER2 3+, Ki-67 40%). The patient underwent palliative VMAT radiotherapy to both brain lesions (10 fractions, 3000 cGy) and was placed on a multidisciplinary team plan for Tykerb plus Xeloda. The multidisciplinary team reviewed the breast cancer brain metastases and planned treatment with Tykerb and Xeloda pending approval. Metastatic breast cancer treatment with Tykerb and Xeloda was initiated, with Xeloda prescribed at 8 × 14 mg; laboratory results were accepted. For cycle 3 of Xeloda, the patient received the dose today, and Tykerb was scheduled with a due date of 10 June 2026. Laboratory values were accepted for reassessment after cycle 3 by PET/CT and brain MRI. The patient has completed three cycles of the Xeloda/Tykerb regimen to date. MRI of the brain demonstrated a regressive course of the lesions and surrounding edema, with the high-parietal lesion decreasing to 1.9 × 0.4 × 2.9 cm (from 1.8 × 3.8 × 4.9 cm) and the occipital lesion decreasing to 2.8 × 2.7 × 1.9 cm (from 4.8 × 3.9 × 3.7 cm). PET/CT was pending. The patient received cycle 4 of Xeloda and Tykerb today; laboratory results were within the accepted range for liver support. The patient has completed four cycles of the Xeloda/Tykerb regimen to date. MRI performed in June 2026 showed regression of residual lesions and surrounding edema. A PET scan confirmed no residual disease in the breast operative bed, no hepatic focal lesions, and no osseous involvement. The patient continued Xeloda 3 × 2 × 14 mg and Tykerb 5 tablets; laboratory values remained acceptable. The patient received Xeloda 5 × 1 × 14 mg today, with bilirubin 1.2 mg/dL supporting liver function, and continued Tykerb 5 × 1 × 30 mg as per follow-up plan.		
تحريراً في ,		
	التاريخ :	18-09-2026
	الوقت :	
	هنا أسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,		



OUTPATIENT CLINICAL SUMMARY

PIN	718526	Visit Date	27/02/2026
Patient Name	FATMA ABDELQADR MOHAMED	Specialty	Oncology
Gender	Female	Company	AXA EGYPT
Birth Date / Age	25/09/1983 / 42Year(s)	Insurance Card	5110189787W1
Nationality	Egyptian		
Marital Status	Married		
National ID	28309251300000		

Chief Complaint

42 yrs female, pre-menopausal, HTN, Fh: brother (cancer bladder)

2023: Rt breast mass bx: IDC grade II, ER 3/8, PR 0/8, HER2 (+3) Ki67= 30%
Rt BCS in 5/2023, received neo-adj HPTC x 6 cycles, then completed Physego for 1 year
Surgical pathology: CR
then PORT, and tamoxifen for 2 yrs then shifted to femara+ zoladex in December 2025
1 month ago: persistent headache and amnesia
MR brain outside : left parieto-temporal & temporal middle cranial fossa two lesions 5.5 cm & 3.3 cm
, with vasogenic edema, midline shift and compressed mid brain.
PET scan (16/12/2025):on system: TWO cerebral avid lesions , NO other distant mets

Underwent surgical excision on 30/12/2025:
Left parietal extra axial SOL, excision: fragmented specimen
No comment on size of tumor excised or margins
Malignant epithelioid neoplasm for immunophenotyping

30/1/26
Immunoprofile is keeping with metastatic carcinoma of breast; Ductal
ER positive (8/8); PR negative (0/8); Her-2 neu (Score 3+) Ki 67 = 40%
for MR brain with contrast, echo, labs, RT consultation
then possible Enhertu
still on femara+ zoladex
left small cervical tender LN for neck US
For tumor board

Received palliative radiotherapy VMAT on both lesions (localized field) 10 fractions
ended 26/2/2026

For Enhertu for 4 cycles then re-evaluation

Physical Exam

Investigation

Other Procedures

Diagnosis

C50 Malignant neoplasm of breast Includes:connective tissue of breast Excludes:skin of breast (C43.5, C44.5)

Treatment Plan

for systemic treatment

Medication

PP09FEMAR1T-FEMARA TAB 2.5MG X 30	
P02BISOC1T-BISOCARD 10MG TAB X30	
P18TIRAT5T-TIRATAM 500MG TABLETS (B30)	
P12DOLIP3T-DOLIPRANE 1 GM TAB(B20)	

Doctor Name : DR MAI ASHRAF MOHAMED GAAFAR

Print Date / Time 2026-02-27 10:17:59

Arabic Notes

[Handwritten signature]

LABORATORY REPORT

Bill No. : 5CR462972

Order No. : OP5 - 516242

Order Date : 09-Jan-2026 19:21

Patient Name : FATMA ABDELQADR ALI MOHAMED

Age/Gender : 42 Year(s)/Female Room : OPD

Hospital No. : EG01.0000718526

Sample Date : 09-Jan-2026 19:21

Doctor Name : Mai Ashraf Mohamed Gaafar [ONC02]

Test Date : 11-Jan-2026 16:06

HISTO-PATHOLOGY

DESCRIPTION	RESULTS	UNITS	REFERENCE RANGE(S)
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DIAGNOSIS

Left parietal extra axial SOL,
excision diagnosed as Malignant
epithelioid neoplasm for
immunophenotyping (# 4173-25):
Immunoprofile is keeping with
metastatic carcinoma of breast;
Ductal
ER positive (8/8); PR negative
(0/8); Her-2 neu (Score 3

DIAGNOSIS

Left parietal extra axial SOL, excision diagnosed as Malignant epithelioid neoplasm for immunophenotyping (# 4173-25):
Immunoprofile is keeping with metastatic carcinoma of breast; Ductal
ER positive (8/8); PR negative (0/8); Her-2 neu (Score 3+) and high Ki 67 proliferative activity of about 40%

CLINICAL INFORMATION :

Left parietal extra axial SOL, excision diagnosed as Malignant epithelioid neoplasm for immunophenotyping (# 4173-25)

MICROSCOPIC EXAMINATION

Sections examined for GATA-3, ER, PR, Her-2 and Ki-67 using Ventana Benchmark ultra autostainer and Ventana ultra view DAB detection system. Immunostaining revealed:

- Positive reaction to ER (90% with strong intensity Allred score 8/8).
- Negative reaction to PR (0% with strong intensity Allred score 0/8).
- Positive complete membranous staining in > 10% of invasive tumor cells; Positive reaction to Her-2 (Score 3).
- Positive reaction to GATA-3
- Ki-67 proliferation index is high (about 40%)

IMMUNOPHENOTYPING PANEL

SAMPLE NO

Specimen Type

COMMENT

The immunohistochemical results
of all received specimens are
verified in single collective report
[Please refer to the final verified
collective report]

Technologist [XL60020]

NOTE: This report is generated by computer. Any manual correction will make it null and void. **Thank You. We Care!!!**

30/2026 14:46:50 PM Cairo 47, Joseph Tito Street, Taha Hussein axis, New Nozha

Alexandria: Mehwer El Taamear km23 - Alex West Compound

القاهرة: ٤٧ شارع جوزيف تيتو تقاطع محور طه حسين النزهة الجديدة
الإسكندرية: الكيلو ٢٣ طريق محور التعمير - كمباوند أليكس ويست



LABORATORY REPORT

Bill No. : 5CR462972

Order No. : OP5 - 516242

Order Date : 09-Jan-2026 19:21

Patient Name : FATMA ABDELQADR ALI MOHAMED

Hospital No. : EG01.0000718526

Doctor Name : Mai Ashraf Mohamed Gaafar [ONC02]

Age/Gender : 42 Year(s)/Female

Room : OPD

Sample Date : 09-Jan-2026 19:21

Test Date : 11-Jan-2026 16:07

HISTO-PATHOLOGY

DESCRIPTION	RESULTS	UNITS	REFERENCE RANGE(S)
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COMMENT

The immunohistochemical results of all received specimens are verified in single collective report [Please refer to the final verified collective report]

MONOCLONAL ANTIBODY: KI 67 (MIB-1)

SAMPLE NO

Specimen Type

COMMENT

The immunohistochemical results of all received specimens are verified in single collective report [Please refer to the final verified collective report]

COMMENT

The immunohistochemical results of all received specimens are verified in single collective report [Please refer to the final verified collective report]

PROGESTERONE (PR RECEPTORS)

SAMPLE NO

Specimen Type

COMMENTS

The immunohistochemical results of all received specimens are verified in single collective report [Please refer to the final verified collective report]

COMMENTS

The immunohistochemical results of all received specimens are verified in single collective report [Please refer to the final verified collective report]

Technologist [XL60020]

OTE: This report is generated by computer. Any manual correction will make it null and void. **Thank You. We Care!!!**

30/2025 4:40:50 PM
30/2025 4:40:50 PM
Cairo: 4675 Joseph Tito Street, Taha Hussein axis, New Nozha

Alexandria: Mehwer El Taamear km23 - Alex West Compound

القاهرة: ٤٧ شارع جوزيف تيتو تقاطع محور طه حسين النزهة الجديدة
الإسكندرية: الكيلو ٢٣ طريق محور التعمير - كمباوند أليكس ويست



Patient Name: FATMA ABDELKADER ALI	Patient ID: EG01.0000718526
Study Date: 14/09/2026 01:08:00 PM	Age: 42 Year(s), 11 Month(s)
Ref. Phy.: Gaafar^Mai^Ashraf Mohamed^^	Sex: Female

MD CT SCAN EXAMINATION OF THE CHEST AFTER DYNAMIC I.V. CONTRAST INJECTION

Clinical History:

- A known case of breast cancer with cerebral metastasis, coming for followup.

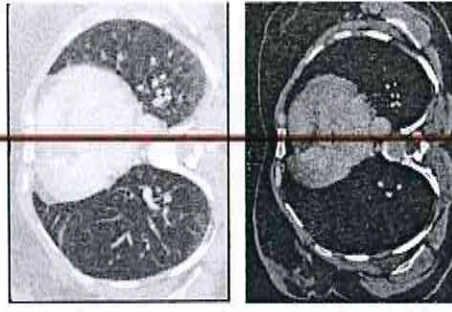
Exposure Dose: 2352μGy.

Findings:

- Diffuse mosaic attenuation pattern of both lung fields in the form of ill-defined areas of subtle GG haziness alternating with intervening areas of reduced parenchymal attenuation, suggesting an element of small airway disease Vs chronic underlying pulmonary vascular pathology, for clinical correlation
- Few bilaterally scattered fine fibroatelectatic bands are seen, likely post-inflammatory fibrotic sequelae.
- Minimal bilateral gravitational lung changes.
- Otherwise, clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- The right breast shows mild diffuse skin thickening with underlying reticulations and operative bed fibrotic stranding, for correlation with sonomammography findings.
- Early dorsal spondylosis.

Opinion:

- Diffuse mosaic attenuation pattern of both lung fields, suggesting an element of small airway disease Vs chronic underlying pulmonary vascular pathology, for clinical correlation
- Few bilaterally scattered fine fibroatelectatic bands, likely post-inflammatory fibrotic sequelae.
- Minimal bilateral gravitational lung changes.
- Right breast changes as described, for correlation with sonomammography findings.
- Early dorsal spondylosis.



Best Regards,
Dr. Salma Tantawy, MD.
Radiology Consultant



المستشفى السعودي الألماني
Saudi German Hospital
 نرعاكم كاهاليينا Caring like family

Patient Name: FATMA ABDELKADER ALI	Age: 42 Year(s), 11 Month(s)
Report Date: 14/09/2026 01:29:58 PM	Patient ID: EG01.0000718526
Ref. Phy.: Gaafar Mai Ashraf Mohamed ^{vv}	

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST

ADMINISTRATION

Clinical History:

- A known case of breast cancer with cerebral metastasis, coming for followup.

Exposure Dose: 2352μGy.

Findings:

- The liver is enlarged (22.5cm at maximum CC dimension) with homogenously reduced CT texture. No focal lesions or contour irregularities.

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.

- No intra-hepatic or extra-hepatic biliary radicles dilatation.

- Normal size & parenchymal texture of the spleen with no focal lesions seen.

- The stomach is of average capacity, showing diffuse mural thickening with thickened gastric folds and rugae, suggestive of mild gastritis, for clinical correlation and endoscopic assessment if warranted.

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

- Normal appearance of the pancreas, aorta, IVC and both suprarenals.

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

- Although the sigmoid colon is inadequately distended, yet it shows minimal diffuse mural thickening suggesting mild inflammatory involvement, for clinical evaluation.

- Diffuse lumbosacral spondylosis.

- A periumbilical anterior midline fat-containing hernia is seen through a defect measuring 2.3cm.

- Normal appearance of the other pelvic organs.

- No abdominal or pelvic masses or collections.

Opinion:

- Fatty hepatomegaly.

- Picture suggestive of mild gastritis, for clinical correlation.

- Periumbilical anterior midline fat-containing hernia.

- Minimal diffuse sigmoid colonic mural thickening, suggesting mild inflammatory involvement, for clinical evaluation.

- Diffuse lumbosacral spondylosis.



Best Regards,

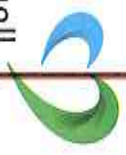
Dr. Salma Tantawy, MD.

Radiology Consultant

Gaipur: 47, Joseph Tito Street, Taha Hussein axis, New Nozha

Alexandria: Mehwer El Taameer km23- Alex West Compound

القاهرة ٤٧ شارع جوزيف تيتو تقاطع محور طه حسين الزرعة الجديدة
 الإسكندرية: الكيلو ٢٣ طريق محور العمير - كمباوند اليكس ويست



المستشفى السعودي الألماني
Saudi German Hospital
نزعكم كأهاليًا Caring like family

Patient Name: FATMA ABDELKADER ALI	Age: 42 Year(s), 11 Month(s)
Report Date: 19/09/2026 08:41:56 AM	Patient ID: EG01.0000718526
Ref. Phy.: Mai Ashraf Mohamed Gaafar	Sex: Female

FOLLOW UP MRI OF THE BRAIN WITH CONTRAST

CLINICAL HISTORY:

- Known case of metastatic right breast cancer, pathologically proven as IDC. Under investigation

TECHNIQUE:

- Sagittal T2WI
- Axial T1, T2 and FLAIR WI
- Coronal T1WI
- Post contrast T1 WIs

FINDINGS:

Compared to previous MRI study dated 8/6/2026, the current study reveals;

- Still noted left parietal craniotomy flap.
- Rather stationary course regarding the size of previously noted left high parietal operative bed heterogeneous SOL, currently measuring about 2.9 x 2.3 x 3.3 cm at its maximum AP, TS & CC dimensions respectively, compared to 3 x 2.1 x 3.2 cm at previous study. It shows progression of the peripheral enhancement with currently noted enhancing soft tissue component measuring about 1.6 x 1.1 x 3.2 cm at maximum AP, TS & CC dimensions respectively. Still noted mild surrounding brain edema and gliosis.



- Progressive course regarding the size and enhancement of previously noted left temporal intra-axial SOL, showing peripheral thick nodular enhancement, currently measuring about 4.6 x 2.8 x 2.6 cm at its maximum AP, TS & CC dimensions, compared to 3.1 x 2.2 x 2.2 cm at its maximum AP, TS & CC dimensions respectively at the previous study. Progressive course of surrounding mild brain edema.
- Mild progressive course regarding the size of previously noted related nodular meningeal /dural enhancement focus/ satellite lesion, currently measures about 1 x 1 cm, compared to 7.4 x 6.5 mm.
- Mild dilatation of related cortical sulci and gyri as well as exvacue dilatation of related posterior horn of left lateral ventricle.
- No newly developed lesions.
- Slightly prominent ventricular system.
- Normal appearance of the brain stem or cerebellar lesions.
- Normal extra axial spaces.

OPINION:

- Comparative study with the last study dated 8/6/2026, with overall progressive status of the previous findings as described above.

Best regards,

Dr. Hadeer Zaher, FEBR, ABRI